

Ponatinib / Asciminib nella LMC – Newsletter interna (ultimi 15 giorni)

Generata il 16/08/2026

Generata il: 2026-08-16

Newsletter interna – sintesi operativa

Messaggi chiave (da leggere in 60 secondi):

- Nel periodo emergono soprattutto temi su **TFR / stop terapia** (n=3).
- Secondo filone: **Dose strategy / consolidamento** (n=2).
- Terzo filone: **Safety e comorbidità** (n=2).

Cosa cambia per noi (azioni pratiche):

- Rivedere/rafforzare **monitoraggio cardiovascolare e interazioni** (es. anticoagulanti) nei pazienti in TKI.
- Valutare criteri interni per **consolidamento / de-escalation / TFR** nei pazienti in deep response.
- Verificare percorso di **test mutazionale** e criteri di switch nelle resistenze (incl. T315I).

Safety radar (segnali/interazioni da monitorare):

- Monitorare evoluzione su **TFR / stop terapia** (nuovi segnali/dati).
- Monitorare evoluzione su **Safety e comorbidità** (nuovi segnali/dati).
- Monitorare evoluzione su **Resistenza / T315I / mutazioni** (nuovi segnali/dati).

Top 5 da leggere subito (ordinati per impatto)

1) Nilotinib in chronic myeloid leukemia: clinical value, real-world challenges, and the potential of nilotinib tablets to improve therapeutic outcomes.

Trial clinico

Expert review of hematology

Score: **19**

Focus clinico: TFR / stop terapia, Dose strategy / consolidamento, Safety e comorbidità, Real-world (utilità pratica)

PMID: 42584922 | 10.1080/17474086.2026.2715450

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42584922/>

- **Tipo di studio/popolazione:** Nilotinib has been shown to be highly effective for the treatment of chronic myeloid leukemia (CML) and advancing patients to treatment-free remission (TFR).
- **Risultato chiave:** However, its efficacy and safety may be compromised by nonadherence due to its twice-daily fasting requirement.
- **Rilevanza clinica:** PubMed search (1966 to July 2026) for papers related to the pathology, etiology, and treatment of CML, pivotal clinical trials and real-world studies for tyrosine kinase inhibitors (TKI) focusing on nilotinib, and recent pharmacokinetic and adherence research, including data on the nilotinib tablet.

2) Targeted nanoparticle-mediated Co-delivery of IFITM3 KO and ponatinib reverses TKI resistance in chronic myeloid leukemia.

Preclinico / laboratorio

Frontiers in pharmacology

Score: 5

Focus clinico: Resistenza / T315I / mutazioni, Dose strategy / consolidamento

PMID: 42582078 | 10.3389/fphar.2026.1852978

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42582078/>

- **Tipo di studio/popolazione:** Although ponatinib can cover the T315I mutation, its clinical application is limited by severe adverse reactions at high doses.
- **Risultato chiave:** *In vitro* experiments confirmed that the optimal ratio (2.5 µg IFITM3-sg3 + 5 µM ponatinib) significantly reversed the drug resistance of K562R cells, promoted apoptosis and inhibited proliferation.
- **Rilevanza clinica:** Mechanistically, this study revealed that IFITM3 mediates CML resistance by interacting with HSPA9 to activate the MET/AKT/BCL2 pathway and that IFITM3 KO can block this pathway and exert a synergistic antiresistance effect with ponatinib.

3) [Sexual and reproductive life of men and women with chronic myeloid leukemia: Opinion and practical considerations of the Fi-LMC group].

Altro

Bulletin du cancer

Score: 5

Focus clinico: TFR / stop terapia, Safety e comorbidità

PMID: 42557142 | 10.1016/j.bulcan.2026.04.002

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42557142/>

- **Tipo di studio/popolazione:** In order to ensure safe outcomes for male and female patients with chronic myeloid leukemia (CML) and their children, the Fi-LMC group (France intergroupe de la LMC) discusses strategies regarding sexuality, contraception, fertility, procreation and breastfeeding.
- **Risultato chiave:** Data on bosutinib, ponatinib and asciminib remain insufficient.
- **Rilevanza clinica:** For patients wishing to breastfeed, TKIs must be discontinued sufficiently prior to delivery.

4) Matching-adjusted indirect comparison between asciminib and flumatinib as first-line treatment for chronic myeloid leukemia in China.

Altro

Translational cancer research

Score: 3

Focus clinico: TFR / stop terapia, Sequencing / linee di terapia

PMID: 42591375 | 10.21037/tcr-2026-1659

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42591375/>

- **Tipo di studio/popolazione:** Asciminib, a novel BCR::ABL1 inhibitor that functions by specifically targeting the myristoyl pocket, has shown superior efficacy and favorable safety and tolerability compared with adenosine triphosphate-competitive tyrosine kinase inhibitors (TKIs) in patients with newly diagnosed chronic myeloid leukemia in chronic phase (CML-CP).
- **Risultato chiave:** Safety analysis showed fewer discontinuations due to AEs at 48 weeks with asciminib (5.5%) than with flumatinib (10.2%), corresponding to a significantly lower risk of discontinuation due AEs (risk ratio: 0.29; 95% CI: 0.10-0.84; P=0.02).
- **Rilevanza clinica:** A robust statistical model indicated that asciminib provides consistently superior efficacy and safety over flumatinib, supporting its value as a first-line treatment option for patients with CML-CP.

Evidenze per domande cliniche

TFR / stop terapia (3)

Score: 0

Mini-riassunto (da abstract, estrattivo):

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Trial clinico

Expert review of hematology

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Dose strategy / consolidamento (2)

Score: 0

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Safety e comorbidità (2)

Score: 0

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Sequencing / linee di terapia (1)

Score: 0

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Real-world (utilità pratica) (1)

Score: 0

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Resistenza / T315I / mutazioni (1)

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